

## Chapter VI

# Modern Eugenics Momentum

*The Institutional Rebuild*

## What Is Being Built

Across the period 2024 to 2026, a cluster of legislative and executive measures has expanded the legal authority of governments in Canada and the United States to detain and to compel treatment of persons with severe substance-use, mental health, and concurrent disorders. The measures are not identical. Each is responding to a different combination of opioid mortality, encampment crisis, and political pressure. The legal frameworks they create differ in their procedural safeguards, their authorizing officials, and their scope. The pattern, taken together, is consistent: legislative authority to compel treatment is expanding rapidly in jurisdictions that are, in the same period, constraining the community-based supports through which voluntary care has historically been delivered.

This chapter documents what has been built. Five jurisdictions are examined: Alberta, Saskatchewan, British Columbia, Ontario, and the United States at the federal level. The chapter then applies the framework Chapter II established — the structural-eugenics analysis — to the documented pattern. The structural argument is the effect argument. It is not the claim that contemporary governments harbour the explicit eugenic intent of the early twentieth century. It is the claim that the pattern of building large secure-treatment infrastructure for predominantly poor, disabled, and disproportionately Indigenous populations, in the absence of equivalent investment in community-based alternatives, is structurally continuous with the institutionalization pattern documented in Chapter I — and that the documented overlap of the affected populations with the populations historically subject to the *Sexual Sterilization Act* is not incidental.

## Alberta: The Compassionate Intervention Act

On April 15, 2025, the Government of Alberta introduced Bill 53, the *Compassionate Intervention Act*. The Act received Royal Assent on May 15, 2025. It establishes a framework for involuntary treatment of persons with severe substance-use or addiction whose conduct is determined to be likely to cause substantial harm to themselves or others within a reasonable time.<sup>1</sup>

The Act's operative provisions authorize adult family members, guardians, healthcare professionals, police officers, and peace officers to apply for an assessment order. A three-person Compassionate Intervention Commission, composed of a lawyer, a physician, and a member of the public, all appointed by the provincial government, reviews applications and issues legally binding treatment decisions. The Act provides for a 72-hour assessment and detoxification period during which a person may be detained

for assessment, with hearings before the full Commission required within that window. Secure treatment orders of up to three months may be issued; community-based treatment orders may extend to six months. Care-plan reviews occur every three weeks during the treatment period. The Act replaces the 2006 *Protection of Children Abusing Drugs Act* (PChAD) for youth.<sup>2</sup>

The Act expands the provincial involuntary-treatment framework beyond the existing *Mental Health Act*, RSA 2000, c M-13. The *Mental Health Act* framework operates against persons with mental disorders meeting an impaired-capacity test. The *Compassionate Intervention Act* operates against persons with severe substance-use or addiction even where the capacity test is met. The substantive evidentiary threshold for involuntary detention and treatment under the new framework is lower than the threshold the existing *Mental Health Act* infrastructure required.<sup>3</sup>

The infrastructure investment associated with the Act has been progressively announced. Budget 2025 allocated \$180 million over three years; Budget 2026 increased the allocation to \$291 million over three years (2026–2029) for the construction of two 150-bed compassionate-intervention centres in Edmonton and Calgary. Construction is to begin in 2026 with the centres expected to open by 2029. Approximately 100 secure beds are being established in existing facilities during the implementation phase. Recovery Alberta operates as the provincial health agency administering the framework.<sup>4</sup>

The professional-and-academic response has been substantial. Eric Adams, a constitutional law professor at the University of Alberta, has stated that the legislation is expected to face Charter challenge once it becomes operational, given its interference with Charter Section 7 rights (life, liberty, security of the person) and Section 9 rights (freedom from arbitrary detention). The Canadian Bar Association — Alberta Branch, in a May 9, 2025 letter, raised concerns including the substantive overlap with existing legislation (the *Personal Directives Act*, the *Adult Guardianship and Trusteeship Act*, the *Youth Criminal Justice Act*), procedural-fairness questions including the ability of the “client” to participate in the assessment process, presumption-of-capacity questions, and consent-and-treatment provisions including authorization of medication administration without consent.<sup>5</sup>

The *Canadian Medical Association Journal* published an ethical analysis applying the Kass framework for public-health interventions to the Act, concluding that the framework presents substantive risks across all three Kass-framework domains — privacy, liberty, and justice — with operational consequences including the potential for disproportionate targeting of structurally marginalized populations including First Nations, Inuit, and Métis individuals. The Alberta Medical Association, in a May 7, 2025 statement, identified operational concerns including post-discharge care, legal safeguards, and the operational interaction with social factors including housing and income. Field Law, a regulatory law firm advising health professionals, published a January 6, 2026 analysis documenting operational tensions between the new framework and the existing *Mental Health Act* infrastructure.<sup>6</sup>

The opioid-crisis context within which the Act has been developed is severe. Alberta documented more than 1,800 opioid-related deaths in 2023 and 1,189 deaths in 2024. The March 2025 Edmonton-zone single-month record reached 87 deaths. Carfentanil involvement reached 68 percent of provincial overdoses and 78 percent of Edmonton overdoses in the January to May 2025 period. Opioid Dependency Program enrollment grew from 3,288 clients in 2018 to 12,299 clients in 2024. The provincial policy

direction during the same period has shifted from harm reduction toward recovery-focused approaches, with supervised-consumption-site closures across the province.<sup>7</sup>

## **Saskatchewan: A Parallel Statute**

On December 5, 2025, Premier Scott Moe’s government in Saskatchewan introduced legislation under the same name as Alberta’s: the *Compassionate Intervention Act*. The Saskatchewan bill was tabled on the final day of the fall legislative sitting. The legislation passed on May 5, 2026. The Act is expected to come into force in fall 2026 after regulations are finalized.<sup>8</sup>

The Saskatchewan framework follows the Alberta architecture in substantive structure. Family members may request treatment through the court system; law enforcement may intervene where substance use puts life at serious risk; medical professionals may refer. The first Compassionate Intervention Assessment Centre will be located in North Battleford. The involuntary inpatient unit will be situated at Saskatchewan Hospital North Battleford, an existing psychiatric facility. The Government of Saskatchewan reports that approximately 150 stakeholders participated in information sessions during the legislation’s development.<sup>9</sup>

The opposition record across the Saskatchewan disability, medical, and civil-rights sector mirrors the Alberta record. The Saskatchewan Medical Association issued a joint statement with other physician organizations identifying capacity concerns: “The healthcare system is not ready for this. Emergency departments and other frontline services are already under severe strain, and the system does not have the capacity required to safely implement involuntary pathways.” The John Howard Society of Saskatchewan, which advocates for incarcerated people, echoed the physicians’ concerns. The Opposition NDP voted against the bill, with leader Carla Beck arguing the province had not put adequate protections in place.<sup>10</sup>

The Government cited support from Saskatoon Tribal Chief Mark Arcand, who noted the framework’s stated focus on culturally responsive care. The endorsement is, on the documented record, one of relatively few from Indigenous leadership; the broader Indigenous-disability and First-Nations health-policy sector has not, in the documented public record, endorsed the framework.<sup>11</sup>

## **British Columbia: Mental Health Act Expansion**

British Columbia has expanded involuntary care under its existing *Mental Health Act* framework rather than through a separate Compassionate Intervention Act. On September 15, 2024, Premier David Eby announced a secure-care framework targeting people with concurrent addiction, mental illness, and acquired brain injuries from toxic-drug poisonings. The provincial framework, on the documented public record, is the most direct response to a provincial overdose crisis that has produced more than 15,000 deaths since 2016, when the province declared a public health emergency.<sup>12</sup>

In summer 2024, the province appointed Dr. Daniel Vigo as British Columbia’s first chief scientific adviser for psychiatry, toxic drugs and concurrent disorders. On March 12, 2025, Dr. Vigo issued clinical guidance specifying that involuntary care under the *Mental Health Act* cannot be used to prevent harmful “risk-taking” by people who use drugs whose behaviour is not related to mental impairment. The guidance

attempts to articulate the boundary between the legitimate medical application of involuntary care for persons whose decision-making capacity is impaired by severe mental illness and the use of involuntary care as a policy response to public-space disorder. The boundary is, in the documented operational practice that has followed, subject to substantive interpretive variation across health authorities and facilities.<sup>13</sup>

The first correctional involuntary-care facility, a 10-bed unit at the Surrey Pretrial Services Centre, opened on April 24, 2025. The first secure housing-and-care facility, Alouette Homes on the grounds of the Alouette Correctional Centre in Maple Ridge, opened in June 2025 with 18 long-term involuntary care beds. More than 400 mental-health beds are being built or modernized across the province — approximately 280 outdated beds modernized and more than 140 new beds added — under the framework. On November 24, 2025, the province introduced *Mental Health Act* amendments operationalizing the framework expansion.<sup>14</sup>

The framework targets, in the language of provincial communications, people with “concurrent mental-health and substance-use challenges, and brain injuries from toxic-drug poisonings.” The triple-condition profile the framework engages overlaps substantively with the documentation in the British Columbia Centre on Substance Use literature on the population most affected by the toxic drug supply — a population the same Centre has identified as inadequately served by the involuntary-care expansion in the absence of comparable expansion in voluntary community-based services.<sup>15</sup>

## **Ontario: The Encampment Frame**

Ontario has not, as of the date of this chapter, passed compassionate-intervention legislation parallel to Alberta’s or Saskatchewan’s. The Ontario framework is moving more slowly, but along a recognizable trajectory. In May 2025, the province announced a study of involuntary addictions treatment for people in the correctional system — in jail, on probation, on parole. The Ontario approach explicitly cites the British Columbia model.<sup>16</sup>

The Ontario opioid context is, in absolute terms, the largest in Canada. The province documented 2,600 or more opioid-related deaths in 2023, a 50 percent increase over 2019. Approximately 48 percent of inmates carry substance-use alerts on their profiles (February 2024 data); approximately 28 percent of inmates are voluntarily on prescribed opioid agonist therapy. The Canadian Mental Health Association — Ontario has identified 1,400 or more encampments across the province and an average of seven deaths per day from drug poisonings, with \$18 billion in 2020 social costs from substance use.<sup>17</sup>

The political pressure for expansion has come substantively from municipal leadership. In October 2024, Ontario Big City Mayors — an organization representing 29 mayors of cities over 100,000 population — collectively asked the province to review mental-health laws and the scope of involuntary treatment. Mayor Alex Nuttall of Barrie and Mayor Cam Guthrie of Guelph have been particularly outspoken. The CMHA Ontario position, by contrast, has been to oppose involuntary-treatment expansion and to ask for \$113 million in the 2025-26 budget for community mental health, including 5,000 additional supportive housing units. The expansion of involuntary treatment infrastructure and the expansion of community supportive-housing infrastructure are being requested by different actors at the same time. The province

has, on the documented record, moved more quickly on the first than on the second.<sup>18</sup>

Ontario does have an existing Bill 53, the *Dignity and Mental Health in Jails Act, 2025*, distinct from Alberta's Bill 53. The Ontario Bill 53 addresses correctional mental-health units, requires that 20 percent of inmate beds in correctional institutions be in mental-health support units, and prohibits cruel, inhumane, or degrading treatment within corrections. It is not, on its face, an involuntary-treatment expansion in the sense documented in Alberta and Saskatchewan, but it operates in the same conceptual space — the provision of mental-health treatment within carceral infrastructure.<sup>19</sup>

## **The United States: Executive Order 14321**

On July 24, 2025, President Donald J. Trump signed Executive Order 14321, titled “Ending Crime and Disorder on America’s Streets.” The Order, which adopts pledges from the Project 2025 blueprint, expands the use of involuntary civil commitments for adults experiencing homelessness and serious mental illness, directs the Department of Justice to evaluate homeless individuals arrested for federal crimes as “sexually dangerous persons,” and ends federal funding for Housing First programs. It directs the Secretary of Health and Human Services and the Secretary of Housing and Urban Development to provide federal grants and legal assistance to state and local governments for “maximally flexible civil commitment, institutional treatment, and step-down treatment standards.”<sup>20</sup>

The legal context is the 1999 United States Supreme Court decision *Olmstead v. L.C.*, 527 U.S. 581 — a twenty-six-year-settled precedent guaranteeing persons with disabilities the right to live in community with necessary support. The Executive Order, in the analysis of the National Alliance to End Homelessness, “defies a 26-year settled legal precedent in *Olmstead v. L.C.*” The 2024 Supreme Court decision in *Grants Pass v. Johnson*, which permits ticketing, fining, and arresting people sleeping on public property, provides the procedural environment within which the Executive Order’s institutionalization framework is being deployed.<sup>21</sup>

The opposition to the Executive Order has been broad. The National Alliance to End Homelessness, the Kaiser Family Foundation in an August 20, 2025 analysis, the law firm Epstein Becker Green in a July 30, 2025 legal review, the Prison Policy Initiative in an August 5, 2025 analysis, and Shannon Minter of the National Center for LGBTQ Rights have each identified substantive concerns. The Epstein Becker Green analysis observes that “the Executive Order does not include access to federal funds to build out treatment capacity for people who are civilly committed” and that “the Trump administration’s efforts so far have involved cutting funds for mental health treatment, not offering to fund increased capacity.”<sup>22</sup>

## The Hari/Rat Park Frame Reapplied

The framework Chapter II established turns on the distinction between policy responses to addiction that operate through isolation and policy responses that operate through connection. The Rat Park studies conducted by Bruce K. Alexander and colleagues at Simon Fraser University in the 1970s established experimentally what the public-health literature has subsequently confirmed: isolated, deprived environments produce predictable adaptive responses, including substance dependence, that are mispathologized as individual failure when the structural conditions producing them are not also examined. The work has been synthesized for general audiences most prominently by Johann Hari, whose central reframe is that the opposite of addiction is not sobriety but connection.<sup>23</sup>

The Portugal decriminalization model, introduced in July 2001, is generally cited as the structural opposite of the mandate-treatment framework. Portugal's framework decriminalizes personal possession of small quantities of drugs and routes persons identified with problematic substance use into voluntary treatment, integrated with housing, employment, and community-reintegration supports. The documented outcomes, across European Monitoring Centre data and the academic literature, include substantial reductions in HIV transmission, overdose deaths, and the incarceration of drug users, alongside documented increases in treatment uptake.<sup>24</sup>

The framework currently being built across Alberta, Saskatchewan, British Columbia, Ontario, and at the United States federal level operates on the opposite premise. The mechanism it deploys is mandatory treatment within secure facilities, often co-located with or adjacent to correctional infrastructure (Surrey Pretrial Services Centre; Alouette Correctional Centre grounds; the correctional-system pathway in Ontario; the Saskatchewan Hospital psychiatric facility), authorized under legal frameworks that lower the evidentiary threshold below the existing mental-health-act standards. The pattern occurs in the same jurisdictions and during the same period in which voluntary community-based supports are being constrained — through supervised-consumption-site closures, harm-reduction funding reductions, the reorientation of addictions services toward abstinence-only models, and (in the United States) the federal defunding of Housing First programs.<sup>25</sup>

The structural argument is direct. A government that responds to a population's isolation by building larger isolation infrastructure, while simultaneously dismantling the community-based mechanisms through which that population's connection could be supported, has chosen the policy response the Rat Park literature identifies as the response that produces the pattern the policy is presented as addressing. The mechanism is recursive. The infrastructure expansion does not address the conditions producing the crisis; it expands the institutional capacity to manage the population the crisis has produced.

## The Parallel to the Provincial Training School Period

Chapter I documented the institutional infrastructure of the Alberta eugenic period: the Provincial Training School at Red Deer, the Alberta Hospital network including Ponoka, Oliver, and Edmonton, the Michener Centre, and the auxiliary facilities through which persons identified as “feeble-minded,” “psychopathic,” or “mentally defective” were institutionalized and, in approximately 2,800 documented cases, sterilized under the authority of the Eugenics Board. The infrastructure was not built on explicit eugenic legislation alone. It was built on a broader consensus that institutional confinement was the appropriate response to populations the surrounding social and economic structure could not or would not accommodate.<sup>26</sup>

The populations the historical institutionalization framework engaged are substantively the same populations the contemporary involuntary-treatment frameworks engage. The overlap is documented across multiple dimensions. The Alberta *Sexual Sterilization Act* period’s disproportionate impact on Indigenous women is paralleled by the CMAJ ethical analysis warning of disproportionate targeting of First Nations, Inuit, and Métis individuals under the *Compassionate Intervention Act*. The historical institutionalization of persons with cognitive disability and mental illness is paralleled by the contemporary framework’s targeting of persons with concurrent mental-health and substance-use conditions — conditions whose presentation frequently overlaps with cognitive disability and acquired brain injury. The historical institutionalization of poor persons whose poverty was characterized as moral failure is paralleled by the contemporary framework’s targeting of unhoused persons whose homelessness is characterized in the United States Executive Order as “crime and disorder.”<sup>27</sup>

The mechanism of authorization differs. The Eugenics Board authorized sterilization. The contemporary Commissions and review boards authorize involuntary treatment and detention in secure facilities. Sterilization is not a contemporary operational tool of the framework. Detention and compelled treatment in secure facilities are.

The structural argument is the effect argument, not the intent argument. The framework that is being built does not require the eugenic intent of the 1928 legislation to produce structurally continuous outcomes for structurally continuous populations. It requires only the operating premise that some populations are properly subjects of institutional management rather than community participants — and the operational capacity to act on that premise at scale.

## What This Chapter Establishes

Five observations follow.

**First**, a coordinated expansion of involuntary-treatment legal authority is occurring across multiple Canadian provinces and at the United States federal level during the period 2024 to 2026. The frameworks differ in detail. The pattern, taken together, is consistent.

**Second**, the expansion is occurring in the same jurisdictions and during the same period in which community-based supports for the same population are being constrained. The Alberta Compassionate

Intervention Act has proceeded alongside supervised-consumption-site closures and the Bill 12 reduction of disability income documented in Chapter IV. The United States Executive Order has proceeded alongside the federal defunding of Housing First. The Saskatchewan and Ontario frameworks are proceeding in the absence of comparable expansion in voluntary community-based services. The pattern is not a single jurisdictional choice. It is a cross-jurisdictional policy direction.

**Third**, the populations the contemporary frameworks engage overlap substantively with the populations the historical institutionalization framework engaged. Persons with concurrent mental-health and substance-use conditions, persons with acquired brain injuries, persons experiencing homelessness, and First Nations, Inuit, and Métis individuals appear disproportionately across the documented analyses of both periods. The overlap is not incidental. It is the documented structural feature of the policy choice.

**Fourth**, the framework operates against the public-health evidence on the conditions producing the crisis the framework is presented as addressing. The Rat Park research tradition, the Portugal decriminalization outcome data, the Housing First evidence base, and the harm-reduction literature each document that connection-based and community-based responses produce better outcomes for the affected populations than isolation-based and institutional responses. The framework currently being built operates in the opposite direction.

**Fifth**, the historical parallel is direct. The Provincial Training School period in Alberta, the parallel institutionalization patterns documented in adjacent jurisdictions, and the contemporary involuntary-treatment expansion all share the operating premise that some populations are properly subjects of institutional management rather than community participants. The mechanism of authorization has changed. The operating premise has not.

The chapter that follows develops the closing argument of the piece. It examines, across all six chapters preceding it, the question the piece's title puts on the record: whether the consent the contemporary frameworks claim to have obtained from the populations they engage was ever the consent the standard requires, or whether consent was the narrative offered to obscure a policy direction that does not, on the documented evidence, require consent to proceed.



## Notes — Chapter VI

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1. Legislative Assembly of Alberta, Bill 53, *Compassionate Intervention Act*, 31st Legislature, First Session; introduced April 15, 2025; Royal Assent May 15, 2025. Government of Alberta, “Compassionate Intervention,” [alberta.ca/compassionate-intervention](https://alberta.ca/compassionate-intervention).
2. *Compassionate Intervention Act*, statutory provisions on assessment-order authority, Commission composition, 72-hour assessment-and-detoxification period, secure-treatment-order duration (up to 3 months), community-based-treatment-order duration (up to 6 months), and care-plan review intervals (every 3 weeks); CBC News, “Alberta introduces controversial involuntary addictions treatment bill,” April 15, 2025, [cbc.ca/news/canada/edmonton/alberta-introduces-controversial-involuntary-addictions-treatment-bill-1.7511051](https://cbc.ca/news/canada/edmonton/alberta-introduces-controversial-involuntary-addictions-treatment-bill-1.7511051).
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4. Government of Alberta, Budget 2025 and Budget 2026, infrastructure allocations for compassionate-intervention centres; Recovery Alberta program documentation; The Alberta Disability System Breakdown, *What Dropped This Week* (Plain Language Edition), May 2026.
5. Statements of Eric Adams (constitutional law, University of Alberta), documented in contemporaneous Canadian legal and media coverage; Canadian Bar Association — Alberta Branch, “Letter re Compassionate Intervention Act,” May 9, 2025, [cba-alberta.org/our-impact/submissions/may-2025-letter-re-compassionate-intervention-act/](https://cba-alberta.org/our-impact/submissions/may-2025-letter-re-compassionate-intervention-act/).
6. Canadian Medical Association Journal, ethical analysis applying the Kass framework to the Compassionate Intervention Act, [cmaj.ca/content/197/40/E1346](https://cmaj.ca/content/197/40/E1346) (citation to be verified before final edition); Alberta Medical Association, “Compassionate Intervention Act: our reaction,” May 7, 2025, [albertadoctors.org/news/publications/presidents-letter-compassionate-intervention-act-our-reaction/](https://albertadoctors.org/news/publications/presidents-letter-compassionate-intervention-act-our-reaction/); Field Law, “Compassion vs. Coercion,” January 6, 2026.
7. Government of Alberta opioid surveillance data, 2023 and 2024 calendar years; The Alberta Disability System Breakdown, *Building Up*, Part One, Section 1.7 (Mental Health and Addiction as a Lifecycle Dimension), May 2026; Global News, “This Isn’t Over Yet: Opioid-Related Deaths Reach New Heights in Edmonton,” July 3, 2025.
8. Government of Saskatchewan, news release, “Legislation Passes for Compassionate Intervention for Addictions Treatment,” May 5, 2026, [saskatchewan.ca/government/news-and-media/2026/may/05/](https://saskatchewan.ca/government/news-and-media/2026/may/05/); CBC News, “Sask. introduces involuntary treatment legislation as fall sitting ends,” December 5, 2025; Global News, “Saskatchewan passes law on involuntary drug treatment,” May 2026.
9. Government of Saskatchewan news release, May 5, 2026; Meridian Source, “New Saskatchewan law would allow compulsory addiction care,” December 8, 2025; CBC News, “Involuntary addiction treatment now legal in Sask. despite medical groups’ warnings,” May 6, 2026.
10. Saskatchewan Medical Association, joint statement with other physician organizations on the Compassionate Intervention Act, 2025-2026; John Howard Society of Saskatchewan, public statements opposing the framework; Saskatchewan NDP, opposition statements documented through legislative record and contemporaneous media coverage.
11. Meridian Source, “New Saskatchewan law would allow compulsory addiction care,” December 8, 2025, reporting the statement of Saskatoon Tribal Chief Mark Arcand.
12. Government of British Columbia, news release, “Province launches secure care for people with brain injury, mental illness, severe addiction,” September 15, 2024, [news.gov.bc.ca/releases/2024PREM0043-001532](https://news.gov.bc.ca/releases/2024PREM0043-001532); The Globe and Mail, “B.C. to introduce involuntary care for people with concurrent addiction, mental disorders, premier says,” September 15, 2024.

13. Government of British Columbia, news release, “B.C. improving care for people with mental-health, substance-use challenges,” March 12, 2025, [news.gov.bc.ca/releases/2025HLTH0015-000202](https://news.gov.bc.ca/releases/2025HLTH0015-000202); CBC News, “B.C. doctors get new guidance on involuntary care for drug users,” March 12, 2025.
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15. British Columbia Centre on Substance Use, “B.C.’s plan for involuntary addiction treatment is a step back in our response to the overdose crisis,” 2024-2025 commentary record.
16. Toronto Today / The Canadian Press, “Ontario eyes involuntary addiction treatment for people in jail, on parole, probation,” May 1, 2025.
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18. Global News, “Ontario mayors ask for mental-health law reviews in wake of opioid, encampment crisis,” October 2024; Canadian Mental Health Association — Ontario, “New legislation neglects to include funding for mental health, addictions supports,” 2025 budget submission.
19. Legislative Assembly of Ontario, Bill 53, *Dignity and Mental Health in Jails Act, 2025*, 44th Parliament, First Session, [ola.org/en/legislative-business/bills/parliament-44/session-1/bill-53](https://ola.org/en/legislative-business/bills/parliament-44/session-1/bill-53).
20. Executive Order 14321, “Ending Crime and Disorder on America’s Streets,” signed July 24, 2025; Federal Register citation to be verified at [federalregister.gov](https://federalregister.gov) before final edition; Stat News, “Trump seeks to make it easier for people with mental illnesses to be involuntarily committed,” July 24, 2025.
21. *Olmstead v. L.C.*, 527 U.S. 581 (1999); National Alliance to End Homelessness, “Statement on Trump Administration’s Executive Order on Homelessness,” July 25, 2025; *City of Grants Pass v. Johnson*, 144 S. Ct. 2202 (2024) (citation format to be verified before final).
22. Kaiser Family Foundation, “A Look at the New Executive Order and the Intersection of Homelessness and Mental Illness,” August 20, 2025, [kff.org/mental-health/a-look-at-the-new-executive-order-and-the-intersection-of-homelessness-and-mental-illness/](https://kff.org/mental-health/a-look-at-the-new-executive-order-and-the-intersection-of-homelessness-and-mental-illness/); Epstein Becker Green (Health Law Advisor), “The Civil Commitment Executive Order: Mixed Messages for Behavioral Health Stakeholders, State and Local Governments,” July 30, 2025; Prison Policy Initiative, “Trump’s executive order targeting unhoused people will leave everyone worse off,” August 5, 2025; Minnesota State Legislators joint statement, July 25, 2025.
23. Alexander, B.K., Coombs, R.B., and Hadaway, P.F. (1978), “The effect of housing and gender on morphine self-administration in rats,” *Psychopharmacology* 58: 175-179 (citation to be verified before final edition); Hari, J., *Chasing the Scream: The First and Last Days of the War on Drugs* (2015).
24. Hughes, C.E., and Stevens, A. (2010), “What can we learn from the Portuguese decriminalization of illicit drugs?,” *British Journal of Criminology* 50(6): 999-1022; European Monitoring Centre for Drugs and Drug Addiction (EMCDDA) country reports, Portugal, 2001-2024 series.
25. The pattern of voluntary-services constraint across the documented jurisdictions: Alberta supervised-consumption-site closures, 2024-2025; Saskatchewan harm-reduction funding constraints; United States Housing First defunding under Executive Order 14321; documented across the sources cited in notes 7, 8, 13, and 22 above.
26. See Chapter I and its associated endnotes for the full record of the Alberta Provincial Training School and the broader institutional infrastructure of the Sexual Sterilization Act period.
27. CMAJ ethical analysis cited at note 6; The Alberta Disability System Breakdown, *Indigenous Disability Alberta Collection*, May 2026, documenting the intersection between historical and contemporary patterns affecting First Nations, Inuit, and Métis individuals.